

CC-001: Angela Morrison — Interview Transcript

Interview Transcript

PROJECT INFORMATION

Project	Home-Care-AI	Stage	Discovery → Stage 1 (Customer Research)
Artifact	2 (Synthetic Interview Data)	Participant ID	CC-001
Persona	Care Coordinator	Next Step	Feed into `/summarize-interview` with other CC interviews

SESSION DETAILS

Date	2026-03-31	Duration	44 min
Interviewer	PM Lead	Setting	Face-to-face
Location	CareBridge office, Angela's desk (sticky notes visible everywhere)		

PART A — Narrative Transcript

Opening & Warm-Up

Q: Tell me about your role --- what does a typical week look like for you?

I coordinate care for sixty-plus clients and manage the schedules for twelve nurses and eight care workers. A typical week: I process about fifteen to twenty schedule changes, handle three to five staff absences, field forty-odd calls from families, run two team briefings, and try to stay on top of compliance documentation. Last week I also had to cover a care worker visit myself because absolutely nobody was available.

Q: What tools does your agency currently use?

A scheduling system that's basically a glorified calendar. The EMR that the nurses hate. WhatsApp groups for everything urgent. Spreadsheets for credential tracking. And sticky notes. I know it's embarrassing, but half the critical knowledge about our clients is on sticky notes around my monitor. Which nurse works well with which patient, who needs a female carer, who gets anxious with new people, who has a dog that bites. None of that is in any system.

Block 1 --- adapted for CC: Staffing Disruptions

Q: Walk me through the last time a staff absence caused a problem.

Last Tuesday. Jenny called in sick at six-thirty AM. She had three visits booked starting at eight. I had forty-five minutes to find someone who was qualified for all three patients, available, lived close enough to get there by eight, and — ideally — had met at least one of those clients before. I made eleven phone calls. I found a replacement for two of the three visits. The third had to be cancelled.

[Probe] What happened with the cancelled visit?

The client's daughter called me at nine AM asking why nobody came. I didn't even know the cancellation hadn't been communicated — I'd been dealing with the other two replacements. So she found out when nobody showed up at the door. Her father had been waiting in his chair, dressed and ready. He gets anxious when the routine changes. She was furious, and she was right to be.

Q: How do you decide who to send as a replacement?

It's all in my head. I know that Mrs. Kim only wants female carers. I know that Arthur Kovacs doesn't like new people but will accept them if they're briefed about not moving his things. I know that Lin Chen will refuse entry to anyone she doesn't recognise. None of this is documented anywhere. If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me.

[Emotional note] Angela said this with dark humour but immediately followed with: 'Actually, that scares me.'

Block 3: Communication & Escalation

Q: When families call you with questions, can you always answer them?

No. Sarah Chen calls me twice a week. 'Did mum's visit happen? Who went? What did they find?' Sometimes I can answer from the scheduling system. Sometimes I have to check the EMR, which takes five minutes of loading and searching. Sometimes the nurse hasn't uploaded their notes yet so I'm guessing. I don't blame Sarah. If it were my mum, I'd call every day.

Block 5: Willingness to Prioritize

Q: If a system could handle vacant visit replacements automatically, what would that change for you?

I'd get my mornings back. The six-thirty AM phone calls are the worst part of my job. If the system could detect the absence, find the best available replacement based on qualifications, proximity, and client preferences, propose it to me for approval, and then notify everyone — the replacement carer, the client, and the family — that would save me forty-five minutes per incident. Three incidents a week, that's over two hours of panicked phone calls I don't have to make. But it has to know the soft stuff. Not just 'who's available' — 'who does this patient trust.' That's what takes me eleven phone calls. The first four calls are finding availability. The next seven are finding the right person.

Wrap-Up

Q: Is there anything I didn't ask?

The compliance audit. It's in six weeks and I'm terrified. I know there are care plans overdue for review. I know there are documentation gaps. I can feel where the holes are but I can't quantify them because the system doesn't flag overdue items. I'd love something that just told me: 'Here are the seventeen things that are out of compliance. Here are the five that are critical.' Give me a list and I'll fix it. Right now I'm guessing and hoping the auditor doesn't find what I haven't found yet.

PART B — Filled Note Template

Participant ID	CC-001
Persona	CC
Date	2026-03-31
Duration (min)	44
Interviewer(s)	PM Lead
KEY JOBS	Schedule 60+ clients across 20 staff. Handle 3-5 absences/week. Field 40+ family calls. Maintain compliance documentation. Preserve institutional knowledge about client preferences. Prepare for compliance audit.

CURRENT SOLUTION	Scheduling calendar system. EMR (slow). WhatsApp for urgent comms. Spreadsheets for credentials. Sticky notes for client preferences. Her memory for everything else. Phone calls (11 per vacant visit). "If I get hit by a bus tomorrow, half the knowledge about our clients walks out the door with me. It's on sticky notes and in my head." "Sarah Chen calls me twice a week asking if her mother's visit happened. Sometimes I can't answer." "But it has to know the soft stuff. Not just 'who's available' — 'who does this patient trust.'"
WILLINGNESS	45 min per vacant visit replacement x 3/week = 2.25 hrs of panicked phone calls. Agency would immediately invest in automation. Key criterion: must include 'soft' matching (trust, preferences, continuity) not just skills + proximity.
EMOTIONAL INTENSITY	Dark humour about bus scenario, immediately followed by genuine fear. Dread about compliance audit. Frustration at being the single point of failure. Empathy for Sarah Chen's calls.
ESCALATION BEHAVIOR	Angela IS the escalation system. Nurses call her, families call her, management calls her. No automated triage. She mentally triages: client safety > family anxiety > compliance documentation. When overwhelmed, compliance slips.
SURPRISE FINDING	Sticky note knowledge: critical client preferences exist nowhere but Angela's desk and memory. Also: the compliance audit fear — she can't quantify her own gaps because the system doesn't flag overdue items. She's managing risk by intuition.
TOOL MENTIONS	Scheduling system (basic calendar). EMR. WhatsApp. Spreadsheets. Sticky notes. Phone.
REFERRAL	Tom Bradley at HomeFirst (smaller agency, same problems at scale). Priya Sharma (clinical view of the same agency).
FOLLOW-UP ACTIONS	[] Interview Tom Bradley (CC-002) [] Investigate 'soft matching' criteria for vacant visit algorithm [] Compliance gap detection as product feature